

ICD - 0-3

Carcinoma, infiltrating duct, NOS 8500/3

Site code: breast, NOS C50.9

12/21/10 lw

Final diagnosis

Breast, right, total mastectomy: Infiltrating ductal carcinoma, Nottingham grade III (of III), forming a 3.1 x 2.3 x 2.1 cm mass in the medial portion of the breast (AJCC pT2). Extensive angiolymphatic invasion is present. The surrounding breast parenchyma shows proliferative fibrocystic changes characterized by sclerosing adenosis with associated calcifications. Nipple and skin are without diagnostic abnormalities. All surgical resection margins including the deep margin, are negative for tumor (minimum tumor free margin, 6 mm, deep margin). Skeletal muscle present and not involved by tumor. See comment.

Lymph nodes, separately submitted right axillary No.1 and No.2, excision: A single (of 2) right axillary lymph node is positive for micrometastatic (1 mm) carcinoma, present on frozen section only (AJCC pN1mi).

Lymph nodes, right axillary, dissection: Multiple (7) right axillary lymph nodes are negative for tumor.

Soft tissue, right axillary low region, excision: Fibroadipose tissue. No lymph node tissue or tumor identified.

Comment: Estrogen: Focally positive, 1-10% nuclear staining
Progesterone: Negative, 0% nuclear staining

ER (Estrogen Receptor) test was developed and its performance characteristics determined by
It has not been cleared or approved by the U.S. Food and Drug Administration.

HER2/neu protein overexpression is negative, score of 0, according to the interpretation guidelines in the FDA-approved HercepTest.

UUID: 75F29A17-B6AF-43B9-BE12-3ABF850F23BA
TCGA-AR-A1AH-01A-PR

Redacted



Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case is (circle): QUALIFIED / DISQUALIFIED		
Reviewer Initials: <i>ML</i>	Date Reviewed: <i>12/21/10</i>	